DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

MEMORANDUM

To: Executive Leadership
From: Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
CC: U.S. Department of Health & Human Services, Office of Inspector General (HHS OIG); Office of General Counsel (OGC); U.S. Office of Special Counsel (OSC) – Emily Drake, Attorney
Date: June 13, 2025
Subject: Unauthorized Use of Administrative Override in PolicyStat – Procedural, Compliance, and Retaliatory Concerns

I. Purpose of This Memorandum

I am writing to formally document serious procedural violations and what appears to be retaliatory misconduct involving unauthorized use of the administrative override function in PolicyStat. Between January 14, 2025, and May 27, 2025, Clinton Pope, Chief Quality Officer, made multiple changes to critical policies without following the mandatory approval processes outlined in our own Policy Management document (Section E.6 of Policy 16791603).

As someone who has dedicated my career to ensuring our organization maintains the highest standards of policy governance and patient safety, I find these violations deeply concerning—not just for their immediate compliance implications, but for what they represent about our commitment to the processes we've established to protect our patients and staff.

II. What Actually Happened

Mr. Pope bypassed our established approval workflows on multiple occasions, making unilateral changes to policies that should have involved multiple reviewers and stakeholders. Here's what I discovered:

Policy Management (ID 16791609):

- **January 14, 2025:** Changed standing-order notification timing from 20 to 30 days
- **March 25, 2025:** Added language stating policies remain enforceable beyond review dates (this directly conflicts with IHM Part 1, Chapter 1 requirements)
- **May 13, 2025:** Changed terminology from "policy" to "document" throughout
- **May 27, 2025:** Added disclaimer about legal advice authority

Quality Management (ID 16791687):

- **May 27, 2025:** Used override to add legal disclaimer and update IHM references

Safety Event Reporting (ID 16969672):

- **March 25, 2025:** Expanded CSA definition to include RCA, FMEA, and PRA
- **May 8, 2025:** Added new language about closed-loop communication

Each of these changes should have gone through our established review process involving subject matter experts, Quality Division representatives, governance committees, senior leadership, and myself as the Policy & Procedure Nurse Consultant.

III. The May 25th Directive – A Concerning Development

What troubles me most is the action taken on **May 25, 2025**, when Mr. Pope issued a directive removing me—the Policy & Procedure Nurse Consultant—from the required approval workflow. This directly violates Section E.6 of Policy 16791603, which clearly states that approval workflows "will include: … f. Policy & Procedure Nurse Consultant."

The timing of this action is particularly concerning. It came after I had documented my objections to these procedural violations and made formal reports to the HHS Office of Inspector General about what I believed to be unlawful activity. The sequence of events suggests this may constitute prohibited retaliation under federal whistleblower protection laws.

Potential Legal Violations:

- **5 U.S.C. § 2302(b)(8)-(9):** Prohibits reprisal against federal employees for lawful disclosures of violations
- **Indian Health Manual Part 1, Chapter 7:** Protects employees participating in governance and compliance activities

By removing a mandatory reviewer after I made protected disclosures, Mr. Pope has not only violated our internal policies but may have engaged in retaliatory conduct that warrants formal investigation.

IV. Why These Standards Matter

I want to be clear about why I'm raising these concerns. The standards we're supposed to follow aren't arbitrary bureaucracy—they exist for critical reasons:

Indian Health Manual Part 1, Chapter 1 requires that policies be properly reviewed and revalidated. The March 25th change undermines this by asserting that expired policies remain enforceable without proper revalidation.

Federal Requirements that we must comply with include:

- **FISMA (44 U.S.C. § 3544):** Requires documented change control for all information systems
- **OMB Circular A-130:** Mandates auditable change management protocols
- **Administrative Procedure Act:** Reinforces procedural transparency requirements
- **Joint Commission Standards:** Require structured, traceable policy development processes

V. The Risks We're Facing

These violations create serious risks for the Phoenix Indian Medical Center:

- **Compromised Policy Integrity:** When we bypass our own governance processes, we undermine the credibility of all our policies
- **Enforcement Challenges:** Policies approved through defective processes may be legally unenforceable
- **Regulatory Exposure:** We're vulnerable to audit findings and enforcement actions
- **Staff Confusion:** When leadership doesn't follow established procedures, it sends mixed messages about the importance of compliance

VI. What Needs to Happen Now

I believe we need to take immediate action to address these issues:

1. **Provide Written Justification:** Explain the authority and reasoning behind each override and the May 25th directive
2. **Conduct Comprehensive Review:** Audit all affected policies for legal, clinical, and operational appropriateness
3. **Restore Proper Workflows:** Reinstate the Policy & Procedure Nurse Consultant role in all approval processes
4. **Reprocess Affected Policies:** Submit all modified content through proper approval channels per Section E.6
5. **Implement Safeguards:** Establish controls to prevent future unauthorized use of override functions
6. **Investigate Potential Retaliation:** Have HR and OGC review the May 25th directive for possible retaliatory conduct
7. **Assess Regulatory Reporting:** Determine if these violations require reporting to IHS, HHS, or other oversight bodies.

VII. Our Policy Is Clear

Policy 16791603 states unambiguously:

"All new and revised documents must be approved through established approval workflows within the electronic repository."

This is not a discretionary process. It is a mandatory compliance mechanism that ensures our policies are accurate, legally sound, and consistently applied. Bypassing these requirements places the entire organization at risk.

VIII. Moving Forward

I'm requesting a formal written response by **June 27, 2025** that addresses:

- Clear justification for each documented action
- Specific remedial measures being implemented
- Concrete steps to prevent similar violations
- Commitment to non-retaliation and full procedural compliance

I want to emphasize that my goal is not to create conflict but to ensure we maintain the highest standards of policy governance that our patients, staff, and regulatory bodies expect from us. As healthcare professionals, we have an obligation to follow the very processes we've established to ensure safe, quality care.

I hope we can work together to resolve these issues promptly and strengthen our policy management systems moving forward.

Respectfully submitted,

Digitally signed by Kevin D. Turnage -S
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CC:

- U.S. Department of Health & Human Services, Office of Inspector General (HHS OIG)
- Office of General Counsel (OGC)
- U.S. Office of Special Counsel (OSC) – Attn: Emily Drake, Esq.